



TOWER HEALTH AND SUBSIDIARIES

Consolidated Financial Statements with Information about Federal Awards
in Accordance with the Uniform Guidance

June 30, 2025

(With Independent Auditors' Reports Thereon)

Federal Entity Identification Number: 23-2201344

TOWER HEALTH AND SUBSIDIARIES

Table of Contents

	Page(s)
Part I – Financial Statements	
Independent Auditors' Report	1–2
Consolidated Financial Statements and Notes to Consolidated Financial Statements	3–41
Schedule of Expenditures of Federal Awards	42
Notes to Schedule of Expenditures of Federal Awards	43
Part II – Reports on Internal Control and Compliance	
Independent Auditors' Report on Internal Control over Financial Reporting and on Compliance and Other Matters Based on an Audit of Financial Statements Performed in Accordance With <i>Government Auditing Standards</i>	44–45
Independent Auditors' Report on Compliance for Each Major Federal Program; Report on Internal Control over Compliance; and Report on the Schedule of Expenditures of Federal Awards Required by the Uniform Guidance	46–48
Part III – Findings	
Schedule of Findings and Questioned Costs	49

PART I
FINANCIAL STATEMENTS



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Independent Auditors' Report

The Board of Directors
Tower Health:

Report on the Audit of the Consolidated Financial Statements

Opinion

We have audited the consolidated financial statements of Tower Health and its subsidiaries (the System), which comprise the consolidated balance sheets as of June 30, 2025 and 2024, and the related consolidated statements of operations and changes in net (deficit) assets, and cash flows for the years then ended, and the related notes to the consolidated financial statements.

In our opinion, the accompanying consolidated financial statements present fairly, in all material respects, the consolidated financial position of the System as of June 30, 2025, and 2024, and the changes in its net (deficit) assets and its cash flows for the years then ended in accordance with U.S. generally accepted accounting principles.

Basis for Opinion

We conducted our audit in accordance with auditing standards generally accepted in the United States of America (GAAS) and the standards applicable to financial audits contained in *Government Auditing Standards*, issued by the Comptroller General of the United States. Our responsibilities under those standards are further described in the Auditors' Responsibilities for the Audit of the Consolidated Financial Statements section of our report. We are required to be independent of the System and to meet our other ethical responsibilities, in accordance with the relevant ethical requirements relating to our audits. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our audit opinion.

Responsibilities of Management for the Consolidated Financial Statements

Management is responsible for the preparation and fair presentation of the consolidated financial statements in accordance with U.S. generally accepted accounting principles, and for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of consolidated financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the consolidated financial statements, management is required to evaluate whether there are conditions or events, considered in the aggregate, that raise substantial doubt about the System's ability to continue as a going concern for one year after the date the consolidated financial statements are issued.

Auditors' Responsibilities for the Audit of the Consolidated Financial Statements

Our objectives are to obtain reasonable assurance about whether the consolidated financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditors' report that includes our opinion. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS and *Government Auditing Standards* will always detect a material misstatement when it exists. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Misstatements are



considered material if there is a substantial likelihood that, individually or in the aggregate, they would influence the judgment made by a reasonable user based on the consolidated financial statements.

In performing an audit in accordance with GAAS and *Government Auditing Standards*, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material misstatement of the consolidated financial statements, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the amounts and disclosures in the consolidated financial statements.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the System's internal control. Accordingly, no such opinion is expressed.
- Evaluate the appropriateness of accounting policies used and the reasonableness of significant accounting estimates made by management, as well as evaluate the overall presentation of the consolidated financial statements.
- Conclude whether, in our judgment, there are conditions or events, considered in the aggregate, that raise substantial doubt about the System's ability to continue as a going concern for a reasonable period of time.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit, significant audit findings, and certain internal control related matters that we identified during the audit.

Other Reporting Required by *Government Auditing Standards*

In accordance with *Government Auditing Standards*, we have also issued our report dated November 7, 2025 on our consideration of the System's internal control over financial reporting and on our tests of its compliance with certain provisions of laws, regulations, contracts, and grant agreements and other matters. The purpose of that report is solely to describe the scope of our testing of internal control over financial reporting and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the System's internal control over financial reporting or on compliance. That report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the System's internal control over financial reporting and compliance.

KPMG LLP

Philadelphia, Pennsylvania
November 7, 2025

TOWER HEALTH AND SUBSIDIARIES

Consolidated Balance Sheets

June 30, 2025 and 2024

(Dollars in thousands)

Assets	2025	2024
Current assets:		
Cash and cash equivalents	\$ 135,437	91,056
Patient accounts receivable	236,560	220,299
Other receivables	38,500	40,529
Receivables from affiliates	7,954	13,804
Medical supplies and inventories	57,687	50,439
Estimated third-party payor receivables	27,039	24,339
Prepaid expenses and other current assets	36,704	36,951
Asset held for sale	25,836	25,836
Assets whose use is limited – required for current liabilities:		
Self-insurance funding arrangements	7,482	7,482
Total current assets	<u>573,199</u>	<u>510,735</u>
Assets whose use is limited:		
Self-insurance funding arrangements	35,913	43,656
Under regulatory requirements	4,500	2,000
By board for capital improvements	59,847	71,522
Investments with donor restrictions	66,277	56,473
Total assets whose use is limited, net of current portion	166,537	173,651
Property, plant and equipment, net	593,294	626,725
Right of use assets	223,750	230,697
Investments in joint ventures	41,785	36,997
Other assets	77,460	70,660
Total assets	<u>\$ 1,676,025</u>	<u>1,649,465</u>

TOWER HEALTH AND SUBSIDIARIES

Consolidated Balance Sheets

June 30, 2025 and 2024

(Dollars in thousands)

Liabilities and Net (Deficit) Assets	2025	2024
Current liabilities:		
Current installments of long-term debt	\$ 50	9,355
Operating lease liabilities – current	24,458	24,554
Finance leases – current	4,765	4,354
Financing obligations – current	12,721	12,472
Accounts payable	105,093	132,503
Estimated third-party payor settlements	3,832	3,674
Current portion of estimated self-insurance costs	28,563	49,751
Accrued expenses	66,203	80,527
Accrued vacation	50,328	51,785
CMS advances – current	—	1,657
Other current liabilities	33,847	36,735
Total current liabilities	329,860	407,367
Long-term debt, net of current portion and unamortized discount/premium and deferred financing costs	1,306,736	1,241,528
Operating lease liabilities, net of current portion	216,641	227,263
Finance leases, net of current portion	16,191	12,986
Accrued pension liabilities	26,391	50,126
Other liabilities	8,090	13,625
Financing obligations, net of current portion and deferred financing costs	127,955	137,437
Estimated self-insurance costs, net of current portion	88,248	68,996
Total liabilities	2,120,112	2,159,328
Net (deficit) assets:		
Without donor restrictions:		
Tower Health and subsidiaries	(497,104)	(559,071)
With donor restrictions	53,017	49,208
Total net deficit	(444,087)	(509,863)
Total liabilities and net deficit	\$ 1,676,025	1,649,465

See accompanying notes to consolidated financial statements.

TOWER HEALTH AND SUBSIDIARIES

Consolidated Statements of Operations and Changes in Net (Deficit) Assets

Years ended June 30, 2025 and 2024

(Dollars in thousands)

	<u>2025</u>	<u>2024</u>
Revenues and other support:		
Net patient service revenue	\$ 1,789,504	1,743,279
Other revenue	<u>239,779</u>	<u>213,907</u>
Total revenues and other support	<u>2,029,283</u>	<u>1,957,186</u>
Expenses:		
Salaries and benefits	1,016,804	1,064,892
Supplies	413,022	350,603
Interest	66,113	55,475
Depreciation	65,066	59,415
Purchased services	271,740	202,696
Repairs and maintenance	56,144	65,169
Other	<u>160,952</u>	<u>176,423</u>
Total expenses	<u>2,049,841</u>	<u>1,974,673</u>
Loss from operations before impairment	(20,558)	(17,487)
Impairment	<u>—</u>	<u>9,588</u>
Loss from operations	<u>(20,558)</u>	<u>(27,075)</u>
Nonoperating gains (losses), net:		
Investment income (losses)	3,806	(1,222)
Gain on early extinguishment of debt	69,911	—
Other gains, net	<u>1,742</u>	<u>303</u>
Nonoperating gains (losses), net	<u>75,459</u>	<u>(919)</u>
Excess (deficiency) of revenues, gains and other support over expenses	<u>\$ 54,901</u>	<u>(27,994)</u>

TOWER HEALTH AND SUBSIDIARIES

Consolidated Statements of Operations and Changes in Net (Deficit) Assets

Years ended June 30, 2025 and 2024

(Dollars in thousands)

	Without donor restrictions	With donor restrictions	Total net deficit
Net (deficit) assets at June 30, 2023	\$ (556,938)	47,278	(509,660)
Changes in net (deficit) assets:			
Deficiency of revenues, gains, and other support over expenses	(27,994)	—	(27,994)
Net in unrealized gains on investments	507	3,075	3,582
Net in realized losses on investments	—	(62)	(62)
Change in pension liability	25,205	—	25,205
Donor restricted contributions	—	6,554	6,554
Net assets released from restrictions	—	(9,533)	(9,533)
Change in beneficial interest in trusts	—	1,895	1,895
Other	149	1	150
Change in net (deficit) assets	<u>(2,133)</u>	<u>1,930</u>	<u>(203)</u>
Net (deficit) assets at June 30, 2024	<u>(559,071)</u>	<u>49,208</u>	<u>(509,863)</u>
Changes in net (deficit) assets:			
Excess of revenues, gains, and other support over expenses	54,901	—	54,901
Net in unrealized gains on investments	303	2,688	2,991
Net in realized gains on investments	—	385	385
Change in pension liability	13,260	—	13,260
Donor restricted contributions	—	6,894	6,894
Net assets released from restrictions	—	(7,834)	(7,834)
Change in beneficial interest in trusts	—	1,676	1,676
Other	(6,497)	—	(6,497)
Change in net (deficit) assets	<u>61,967</u>	<u>3,809</u>	<u>65,776</u>
Net (deficit) assets at June 30, 2025	<u>\$ (497,104)</u>	<u>53,017</u>	<u>(444,087)</u>

See accompanying notes to consolidated financial statements.

TOWER HEALTH AND SUBSIDIARIES

Consolidated Statements of Cash Flows

Years ended June 30, 2025 and 2024

(Dollars in thousands)

	<u>2025</u>	<u>2024</u>
Cash flows from operating activities:		
Change in net deficit	\$ 65,776	(203)
Adjustments to reconcile change in net deficit to net cash used in operating activities:		
Unrealized (gains) losses on investments and change in beneficial interest in trusts	(2,043)	2,004
Amortization of bond discount	602	—
Amortization of bond premium	(2,221)	(6,282)
Amortization of deferred financing costs	1,684	487
Change in pension liability, net	(23,735)	(35,403)
Depreciation	65,066	59,415
Impairment	—	9,588
Investment income	(4,088)	(4,904)
Equity in income of affiliates	(10,731)	(18,799)
Gain on disposal of fixed assets	(5,902)	(212)
Contributions and investment income from donor-restricted contributions	(6,894)	(6,554)
Gain on early extinguishment of debt	(69,911)	—
Change in cash due to changes in operating assets and liabilities:		
Receivable from patients and others	(14,665)	(44,079)
Medical supplies and inventories	(7,248)	(4,687)
Prepaid expenses and other assets and other long-term liabilities	(276)	(5,458)
Accounts payable, other liabilities and net change in operating leases	(51,318)	6,282
Estimated self-insurance costs	(1,936)	43,167
CMS advances	(1,657)	1,657
Net cash used in operating activities	<u>(69,497)</u>	<u>(3,981)</u>
Cash flows from investing activities:		
Acquisition of property, plant and equipment	(43,917)	(25,567)
Proceeds from the sale of property, plant, and equipment	10,313	465
Investment in equity investees	—	(4,267)
Distribution from equity investees	5,943	5,404
Sales of investments and assets whose use is limited, net	53,863	9,154
Purchases of investments and assets whose use is limited, net	<u>(40,619)</u>	<u>(13,758)</u>
Net cash used in investing activities	<u>(14,417)</u>	<u>(28,569)</u>
Cash flows from financing activities:		
Contributions and investment income for donor-restricted contributions	6,894	6,554
Repayments related to debt extinguishment	(1,083,282)	—
Proceeds from long-term debt issuance	1,233,030	—
Repayments of long-term debt	(12,055)	(8,680)
Payments of deferred financing costs	(19,907)	(3,130)
Payments on finance leases	(3,356)	(3,793)
Extinguishment of finance leases	(1,020)	—
Proceeds from finance leases	<u>7,991</u>	<u>—</u>
Net cash provided by (used in) financing activities	<u>128,295</u>	<u>(9,049)</u>
Net increase (decrease) in cash and cash equivalents	44,381	(41,599)
Cash and cash equivalents:		
Beginning of year	<u>91,056</u>	<u>132,655</u>
End of year	\$ <u>135,437</u>	<u>91,056</u>
Supplemental cash flow information:		
Cash paid during the year for interest	\$ 63,098	55,874
Fixed asset additions included in accounts payable and accrued expenses at June 30	3,474	11,386

See accompanying notes to consolidated financial statements.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(1) Organizational Structure and Nature of Operations

Tower Health (Parent) is a tax-exempt not-for-profit corporation organized under Section 501(c)(3) of the Internal Revenue Code. The Parent is located in West Reading, Pennsylvania, and provides inpatient, outpatient and emergency care for residents of the greater Berks, Montgomery, and Chester County areas through its subsidiaries (collectively, the System).

(a) *Subsidiaries of the Parent*

Reading Hospital (Hospital) is a tax-exempt not-for-profit corporation providing acute and post-acute care.

Chester/Montgomery hospitals (CMP) are tax-exempt not-for-profit corporations providing acute and post-acute care. These three hospitals include: Brandywine Hospital in Coatesville; Phoenixville Hospital in Phoenixville and Pottstown Memorial Medical Center in Pottstown (collectively, Chester/Montgomery). Brandywine Hospital in Coatesville was sold to Vantage Bridge, LLC in March 2025 for net proceeds of \$10,289, resulting in a gain of \$6,125, which is included within Other revenue in the accompanying statements of operations and changes in net (deficit) assets.

Tower Health Medical Group (THMG) is a tax-exempt entity established to assure access to high quality primary care physicians and specialty physicians in sufficient numbers to meet the community needs for charitable, educational, and scientific purposes. THMG also recruits physicians and provides administrative services for the Hospital, including supervision and instruction for medical students completing their residency training. The Chester/Montgomery clinics and practices are part of THMG.

Tower Health Partners (THP) a Pennsylvania limited liability company, was formed to develop a physician network working in conjunction with the Parent to implement a clinical integration program. Clinical integration is the implementation of an active and ongoing program to evaluate and modify practice patterns by the network's physician participants and create a high degree of interdependence and cooperation among the physicians to control costs and improve the quality and efficiency of health care in the community.

Reading Hospital Foundation (RHF) is a not-for-profit corporation created to support research, education, innovation and fund-raising in support of the Parent and its subsidiaries, and the community. The Parent is the sole member of RHF.

Tower Health Enterprises (THE) is a Pennsylvania limited liability company, was formed to hold the interest in joint ventures.

Also included is Tower Health Urgent Care, Tower Health at Home and Tower Direct, an inhouse ambulance service comprised of BH Trans, Pottstown Ambulance and Southern Berks Regional EMS.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(b) Other Noncontrolled Entities

The following entities are recorded under the equity method of accounting and are included in investments in joint ventures on the accompanying consolidated balance sheets and had the following balances at June 30, 2025 and 2024.

	<u>2025</u>	<u>2024</u>
Spring Ridge	\$ 314	669
Quest	597	453
AHP	286	275
THE – JV	2,662	2,922
AHGPO	5,102	4,277
Acadia	21,959	17,884
Equus	10,865	10,517
STC	—	—
	<u>\$ 41,785</u>	<u>36,997</u>

The Reading Hospital SurgiCenter at Spring Ridge, LLC (Spring Ridge), a limited liability company, was established to provide ambulatory surgery services to the surrounding community. The Hospital maintains a 50% ownership and during the years ended June 30, 2025 and 2024, the Hospital received distributions of \$734 and \$661, respectively.

The Parent is a 20% owner of Quest Behavioral Health, Inc. (Quest). Quest is a not-for-profit corporation providing full service managed behavioral healthcare.

AllSpire Health Partners, LLC (AHP) is an alliance of six systems primarily in New Jersey and Pennsylvania of which the Parent is a 17% owner. The consortium will carry out joint activities in traditional areas of patient care services, research, and education to enhance the value of health care that communities receive.

THE holds interests in various joint ventures including USRC Montgomery Partners, LLC, Commonwealth Cyberknife, LLC, and Pottstown Medical Specialist, Inc., collectively referred to as THE Joint Ventures (THE – JV). During the years ended June 30, 2025 and 2024, THE received distributions of \$47 and \$147, respectively.

AllSpire Health GPO, LLC (AHGPO) is an alliance of the six systems that are part of AllSpire Health Partners, LLC of which the Parent is a 17% owner. The alliance was created to help manage expenses by establishing group purchasing volumes, streamlining suppliers, and implementing efficiencies across all partners. The goal is to identify clinical optimization and revenue opportunities to provide access to quality products to providers and patients. During the years ended June 30, 2025 and 2024, the Parent received distributions of \$4,669 and \$2,192, respectively.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Tower Behavioral Health (Acadia) is a 144-bed inpatient facility and offers inpatient psychiatric care for children, adolescents, adults, and seniors. In addition, Tower Behavioral Health provides outpatient services in a new 30,000 square foot Medical Office Building on the campus for the partial hospitalization program (PHP); intensive outpatient program (IOP); dual diagnosis services; outpatient counseling; and physician services and consultations. The Reading hospital maintains a 29% equity in the joint venture with Acadia Healthcare. During the years ended June 30, 2025 and 2024, the Parent received distributions of \$493 and \$2,265, respectively.

Equus Member Medical School, LLC (Equus), a limited liability company, is a joint venture between Tower Health and Equus Capital Partners of Newton Square, and was established to provide for the construction, operation, and governance of the Drexel Medical School. Tower sublets four of the six floors to the Drexel Medical School, with the remaining two reserved for future use. Tower Health maintains a 61.75% ownership interest and during the years ended June 30, 2025 and 2024, THE received distributions of \$0 and \$139, respectively.

STC Healthcare Partners LLC (STC) is the parent company of St. Christopher's Hospital for Children and the physician company, STC Pediatrics, LLC. STC is owned by the System and Drexel University in equal shares. St. Christopher's is a 188-bed facility in North Philadelphia. The STC investment is recorded under the equity method of accounting. As of June 30, 2025 and 2024, the investment in STC balance is \$0 and is included in investments in joint ventures on the accompanying 2025 and 2024 consolidated balance sheets.

(2) Summary of Significant Accounting Policies

Basis of Accounting

The accompanying consolidated financial statements have been prepared on the accrual basis of accounting in conformity with U.S. generally accepted accounting principles (GAAP). The significant accounting policies followed by the System are as follows:

(a) Principles of Consolidation

The consolidated financial statements of the System include the accounts of the Parent, the Hospital, CMP, THMG, THP, RHF, and THE for the years ended June 30, 2025 and 2024. All entities where the Parent, Hospital, or THE exercise significant influence but for which they do not have control are accounted for under the equity method of accounting. All significant intercompany balances and transactions have been eliminated.

(b) Use of Estimates

The preparation of consolidated financial statements in conformity with GAAP requires management to make estimates and assumptions that affect the reported amounts of assets and liabilities and disclosure of contingent assets and liabilities at the date of the consolidated financial statements and the reported amounts of revenues and expenses during the reporting period. Actual results could differ from those estimates.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(c) *Excess (Deficiency) of Revenues, Gains and Other Support over Expenses*

The consolidated statements of operations and changes in net (deficit) assets include the excess (deficiency) of revenues, gains, and other support over expenses. Changes in net (deficit) assets without donor restrictions that are excluded from this performance indicator, consistent with industry practice, include changes in unrealized gains (losses) on marketable securities classified as other than trading securities, adjustments for defined benefit and other postretirement benefits, and contributions of long-lived assets (including assets acquired using contributions, which by donor restriction were to be used for the purposes of acquiring such assets).

(d) *Cash and Cash Equivalents*

Cash and cash equivalents include investments in highly liquid debt instruments with an original maturity of three months or less. At June 30, 2025 and 2024, the System had cash balances in financial institutions that exceeded federal depository insurance limits. Management believes that the credit risk related to these deposits is minimal.

(e) *Net Patient Service Revenue*

The System's net patient service revenue is reported at the amount that reflects the consideration to which the System expects to be entitled in exchange for providing patient care. These amounts are due from patients, third-party payors, and others and include an estimate of variable consideration for retroactive revenue adjustments due to settlement of audits, reviews, and investigations. Generally, the System bills the patients and third-party payors several days after the services are performed and/or the patient is discharged from the facility.

Revenue is recognized as performance obligations are satisfied. Performance obligations are determined based on the nature of the services provided by the System. Revenue for performance obligations satisfied over time is recognized based on actual charges incurred in relation to total expected (or actual) charges. The System believes that this method provides a reasonable representation of the transfer of services over the term of the performance obligation based on the inputs needed to satisfy the obligation. Generally, performance obligations satisfied over time relate to inpatient services. The System measures the performance obligation from admission into the hospital to the point when it is no longer required to provide services to that patient, which is generally at the time of discharge. Revenue for performance obligations satisfied at a point in time is recognized when goods or services are provided, and the System does not believe it is required to provide additional goods or services to the patient.

The majority of the System's services are rendered to patients with third party coverage. Reimbursement under these programs for all payors is based on a combination of prospectively determined rates, discounted charges, and historical costs. Amounts received under Medicare and Medicaid programs are subject to review and final determination by program intermediaries or their agents. The contracts the System has with commercial payors also provide for retroactive audit and review of claims. Agreements with third-party payors typically provide for payments at amounts less than established charges. Generally, patients who are covered by third-party payors are responsible for related deductibles and coinsurance, which vary in amount. The System also provides services to uninsured patients, and offers those uninsured patients a discount, either by policy or law, from

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

standard charges. The System estimates the transaction price for patients with deductibles and coinsurance and from those who are uninsured based on historical experience and current market conditions. The initial estimate of the transaction price is determined by reducing the standard charge by any contractual adjustments, discounts, and implicit price concessions. Implicit price concessions are determined based on historical collection experience. Subsequent changes to the estimate of the transaction price are generally recorded as adjustments to patient service revenue in the period of the change and are accrued on an estimated basis in the period the related services are rendered and adjusted in future periods as final settlements are determined.

Consistent with the System's mission, care is provided to patients regardless of their ability to pay. The System has determined it has provided implicit price concessions to uninsured patients and patients with other uninsured balances (e.g., copays and deductibles). The implicit price concessions included in estimating the transaction price represent the difference between amounts billed to patients and the amounts the System expects to collect based on its collection history with those patients. Patients who meet the System's criteria for charity care are provided care without charge or at amounts less than established rates. The System has determined that it has provided sufficient implicit price concessions for these accounts. Price concessions consisting of charity care are not reported as revenue.

The System's estimate of the transaction price includes estimates of price concessions for such items as contractual allowances, charity care, potential adjustments that may arise from payment and other reviews, and uncollectible amounts, which are determined using a portfolio approach as a practical expedient to account for patient contracts as collective groups rather than individually. Estimates for uncollectible amounts are based on the aging of the accounts receivable, historical collection experience for similar payors and patients, current market conditions, and other relevant factors.

Subsequent changes to the estimate of the transaction price are generally recorded as adjustments to net patient service revenue in the period of the change. Subsequent changes that are determined to be the result of an adverse change in the payor's or patient's ability to pay are recorded as bad debt expense. Bad debt expense for the years ended June 30, 2025 and 2024 was not significant to the consolidated financial statements.

The percentages of net patient service revenue from patients and third-party payors for the years ended June 30, 2025 and 2024 were as follows:

	2025	2024
Medicare	20 %	21 %
Medicare Advantage	16	15
Medicaid	1	1
Managed Medicaid	8	7
Blue Cross	28	27
Commercial	18	19
Self-pay and other	9	10
	<u>100 %</u>	<u>100 %</u>

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(f) Accounts Receivable

The System has agreements with third-party payors that provide for payment at amounts different from its established rates. Payment arrangements include prospectively determined rates per discharge, reimbursed costs, discounted charges, and per diem payments. Management regularly reviews accounts and contracts and provides appropriate contractual allowances and discounts that are netted against patient accounts receivable in the consolidated balance sheets. The System grants credit without collateral to its patients, most of whom are local residents and are insured under third-party payor arrangements. The percentages of net patient accounts receivable from patients and third-party payors, net of implicit price concessions, as of June 30, 2025 and 2024 were as follows:

	<u>2025</u>	<u>2024</u>
Medicare	13 %	17 %
Medicare Advantage	18	19
Medicaid	2	1
Managed Medicaid	8	8
Blue Cross	19	15
Commercial	32	32
Self-pay and other	8	8
	<u>100 %</u>	<u>100 %</u>

(g) Assets Whose Use is Limited

Assets whose use is limited includes designated assets set aside by the Board of Directors for future capital improvements, assets held by trustees under indenture agreements and self-insurance trust arrangements. The Board of Directors retains control over Board-designated assets and may at its discretion subsequently use these assets for other purposes.

Assets whose use is limited includes cash and cash equivalents, marketable securities (including U.S. government and government agencies, corporate, state, and local government), marketable equity securities (including common, preferred, and foreign stock), exchange traded/listed mutual funds (including fixed income funds), hedge funds, private equity funds, and limited partnerships.

(h) Investments and Investment Income

Investments in equity securities with readily determinable fair values and all investments in debt securities are recorded at fair value in the consolidated balance sheets.

Investment income earned on securities (interest and dividends) is reported in the nonoperating gains (losses) section of the consolidated statements of operations and changes in net (deficit) assets within investment income. Realized gains or losses related to the sale of investments, impairment losses on other than trading investments, and unrealized gains or losses on alternative investments and other board designated investments, are included in the nonoperating gains (losses) section of the consolidated statements of operations and changes in net (deficit) assets in investment income unless the income or loss is restricted by donor or law.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Restricted investments and assets held for self-funding arrangements are classified as other than trading, and net unrealized gains on these instruments are included in the consolidated statements of changes in net (deficit) assets. Impairment losses are included in the consolidated statements of operations and changes in net (deficit) assets within nonoperating gains (losses) as other than temporary impairment on other than trading investments.

The fair value option for financial assets and liabilities permits the System to elect to measure eligible items at fair value on an instrument-by-instrument basis. If elected, this option requires the System to report the unrealized gains and losses on these instruments as part of the performance indicator. Once elected, the fair value option is irrevocable for that instrument. Alternative investments include investments in managed funds, which include hedge funds, private equities, limited partnerships, and other investments that do not have readily determinable fair values and may be subject to withdrawal restrictions. Investments in hedge funds, private equities, limited partnerships, and other investments in managed funds (collectively Alternative Investments) are accounted for at fair value. The unrealized gains or losses from these Alternative Investments are included in the consolidated statements of operations and changes in net (deficit) assets as part of nonoperating gains (losses) within investment income.

(i) Fair Value Measurements

The System estimates fair value based on Financial Accounting Standards Board (FASB) Accounting Standards Codification (ASC) 820, Fair Value Measurement, which uses a fair value hierarchy that prioritizes the inputs to valuation techniques to measure fair value. The hierarchy gives the highest priority to quoted prices in active markets for identical assets or liabilities (Level 1 measurements) and the lowest priority to unobservable inputs (Level 3 measurements). The three levels of the fair value hierarchy are described below:

Level 1: Quoted prices in active markets that are accessible at the measurement date for identical assets and liabilities.

Level 2: Inputs, other than quoted prices in active markets, that are observable either directly or indirectly in which the fair value is determined through the use of models or other valuation methodologies.

Level 3: Unobservable inputs that are supported by little or no market activity and that are significant to the fair value of the assets or liabilities; Level 3 assets and liabilities include financial instruments whose value is determined using pricing models, discounted cash flow methodologies, or similar techniques, as well as instruments for which the determination of fair value requires significant management judgement or estimation.

In addition, the System also estimates the fair value of investments in certain investment companies for which the investment does not have a readily determinable fair value by using net asset value (NAV) per share or its equivalents as a practical expedient.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(j) Property, Plant and Equipment

Property, plant, and equipment are carried at cost, less accumulated depreciation. Expenditures that substantially increase the useful lives of existing assets are capitalized. Routine maintenance and repairs are expensed as incurred. Depreciation is computed using the straight-line method over the estimated useful lives of each class of depreciable asset. Useful lives range as follows:

Land improvements	5–25 years
Buildings and building improvements	10–50 years
Fixed equipment	5–15 years
Movable equipment (including software and hardware)	3–15 years

Gains and losses resulting from the retirement or sale of property, plant and equipment are included in the consolidated statements of operations and changes in net (deficit) assets. Interest cost incurred on borrowed funds during the period of construction of capital assets is capitalized as a component of the cost of acquiring those assets. Gifts of long-lived operating assets such as land, buildings or equipment are reported as contributions without donor restrictions and are excluded from the performance indicator unless explicit donor stipulations specify how the donated asset must be used. Gifts of long-lived assets with explicit donor restrictions that specify how the assets are to be used and gifts of cash or other assets that must be used to acquire long-lived assets are reported as support with donor restrictions. Absent explicit donor stipulations about how long those long-lived assets must be maintained, expirations of donor restrictions are reported when the donated or acquired long-lived assets are placed in service.

The System routinely evaluates the carrying value of its property, plant and equipment for impairment whenever events or changes in circumstances indicate that the carrying amount of assets, or related group of assets, may not be recoverable. The triggering event for impairment testing of long-lived assets, primarily property, plant and equipment, was evaluated and impairment testing was carried out as of June 30, 2024. There was no triggering event as of June 30, 2025. The System measures the fair value of its various asset groups to determine whether the fair value exceeds the carrying value based upon a market-based approach, which included consideration of the multiple indications of interest received from the various independent third parties. The market-based approach and related analysis utilizes values observed in private market transactions and offers. The asset groups identified for closure were written down to salvage value. The fair value of certain asset groups was less than the carrying amount and accordingly, an impairment was necessary. This resulted in an impairment of long-lived assets charge of \$0 and \$9,588, which was recorded as a noncash operating expense within impairment in the consolidated statements of operations and changes in net (deficit) assets for the years ended June 30, 2025 and 2024, respectively.

(k) Deferred Financing Costs

Deferred financing costs are amortized over the period the debt or financing obligations are outstanding using the straight-line method, which approximates the effective interest method. Amortization of deferred financing costs totaled \$1,684 and \$487 for the years ended June 30, 2025 and 2024,

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

respectively. Deferred financing costs totaled \$22,049 and \$10,883 as of June 30, 2025 and 2024, respectively. As of June 30, 2025, the System incurred \$19,907 in new deferred financing costs and had write-offs of \$7,057 in deferred financing costs.

(l) *Estimated Self-insurance – Costs*

The provision for estimated self-insured claims includes estimates of the ultimate costs for both reported claims and claims incurred but not reported. The System self-insures – its medical malpractice, general liability, and workers' compensation risks. Reserve estimates are subject to the impact of changes in claim trends as well as prevailing social, economic, and legal conditions. The ultimate net cost of settling these liabilities may vary from the estimated amounts. Accordingly, reserve estimates are continually reviewed and updated, and any resulting adjustments are reflected in the performance indicator.

(m) *Accrued Vacation*

The System records a liability for amounts due to employees for future paid leave, which are attributable to services performed in the current and prior periods.

(n) *Bond Premiums and Discounts*

Bond premiums and discounts are amortized to interest and expensed as direct additions or reductions of the carrying values of the related debt instruments from which the discounts or premiums arose. Bond premiums and discounts are amortized to interest expense over the period during which the debt is outstanding using the straight-line method, which approximates the effective interest method.

(o) *Net Assets with Donor Restrictions*

Net assets with donor restrictions are those whose use by the System have been limited by donors to a specific time period or purpose and net assets that have been restricted by donors to be maintained by the System in perpetuity.

From time to time, the fair value of assets associated with individual donor-restricted endowment funds may fall below the level that the donor requires the System to retain as a fund of perpetual duration. In accordance with GAAP, deficiencies of this nature are to be reported in net assets with donor restrictions as of year-end. These deficiencies can result from unfavorable market fluctuations that occur shortly after the investment of new donor restricted contributions and continued appropriation for certain programs that was deemed prudent by the Board of Directors. No material deficiencies existed at June 30, 2025 and 2024.

(p) *Other Revenue*

Significant components of other revenue include rental income on leased properties, residential revenue, tuition revenue for The Reading Hospital School of Health Sciences, contributions without donor restriction, and cafeteria revenues.

Additionally, pharmacy sales and other contracts related to health care services are included in other revenue and consist of contracts which vary in duration and in performance. Revenue is recognized

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

when the performance obligations identified within the individual contracts are satisfied and collections can be reasonably assured.

(q) Donor Restricted Gifts

Unconditional promises to give cash and other assets are reported at fair value at the date the promise is received. The gifts are reported as donor restricted support if they are received with donor stipulations that limit the use of the donated assets. When a donor restriction expires, that is, when a stipulated time restriction ends and/or purpose restriction is accomplished, net assets with donor restrictions are reclassified as net assets without donor restrictions and reported in the consolidated statements of operations and changes in net (deficit) assets as net assets released from restrictions and in other revenue.

Donor-restricted contributions whose restrictions are met within the same year as received are reported as contributions without donor restrictions in the accompanying consolidated statements of operations and changes in net assets.

(r) Income Taxes

The System is a not-for-profit corporation as described in Section 501(c)(3) of the Internal Revenue Code and is exempt from federal income taxes on related income pursuant to Section 501(a) of the Code. On such a basis, the exempt entities do not incur liability for federal income taxes, except in the case of unrelated business income.

The System evaluates uncertain tax positions using a two-step approach for recognizing and measuring tax benefits taken or expected to be taken in an unrelated business activity tax return and disclosures regarding uncertainties in tax positions. No adjustments to the consolidated financial statements were required as a result of this evaluation.

(s) Uncompensated Care and Community Service

The System provides services to patients who meet the criteria of its charity service policy without charge or at amounts less than the established rates. Criteria for charity care consider the patient's family income, family size, and ability to pay. Individuals who qualify for charity care do not have insurance or other coverage.

The System maintains records to identify and monitor the level of charity care and community service it provides. These records include the amount of charges foregone based on established rates for services, and supplies furnished under its charity care and community service policies, and the estimated cost of those services.

Charges foregone for uncompensated care as determined in accordance with the System's policies were approximately \$74,068 and \$46,599 in the years ended June 30, 2025, and 2024, respectively. Direct and indirect costs to provide these services were approximately \$16,235 and \$11,119 for the years ended June 30, 2025 and 2024, respectively. The estimated costs were based on a calculation, which multiplied the cost to charge ratio by the gross charges associated with providing uncompensated care to patients. The cost to charge ratio was obtained from the System's most recently filed Medicare cost report.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Additionally, the System sponsors certain other service programs and charity services, which provide substantial benefit to the broader community. Such programs include services to needy populations requiring special services and support, community service programs and charity services, as well as health promotion and education.

The System's community service includes the Medical Assistance program, which makes payment for services provided to families with dependent children, the aged, the blind, and the permanently and totally disabled, whose income and resources are insufficient to meet the costs of necessary medical services. Payments from the Medical Assistance program are generally less than the System's cost of providing the service.

In addition, community service represents the cost to deliver services to the community, net of any payment received for those services. Included in these services are the System's subsidies of outpatient clinics, education of medical professionals who work with various health care providers in the community upon graduation and community mental health programs. The System also sponsors health fairs and other wellness programs throughout the community.

(t) Recent Accounting Pronouncements

From time to time, new accounting guidance is issued by the FASB or other standard setting bodies that is adopted by the System as of the effective date or, in some cases where early adoption is permitted, in advance of the effective date. The System has assessed the recently issued guidance that is not yet effective and, unless otherwise indicated, believes the new guidance will not have a material impact on its consolidated financial position, results of operations, or cash flows.

(3) Net Patient Service Revenue

The System has agreements with third-party payors that provide for payments at amounts different from its established rates. A summary of the payment arrangements with major third-party payors follows.

(a) Medicare and Medicare Advantage

Inpatient acute care and rehabilitation services rendered to Medicare program beneficiaries are paid at prospectively determined rates per discharge. These rates vary according to a patient classification system that is based on clinical, diagnostic, and other factors. Outpatient services are reimbursed by Medicare under the Ambulatory Payment Classification System. The System is reimbursed for cost reimbursable items at a tentative rate with final settlement determined after submission of annual cost reports by the System and audits thereof by the Medicare fiscal intermediary. The System's classification of patients under the Medicare program and the appropriateness of their admission are subject to medical necessity reviews by independent organizations under contract with the Center for Medicare and Medicaid Services (CMS). The System has received settlements on Medicare cost reports through June 30, 2022.

(b) Medicaid and Managed Medicaid

On December 29, 2010, the Pennsylvania Department of Human Services (DHS) received approval from the Centers for Medicare & Medicaid Services for the state plan amendments pursuant to Act 49 of 2010, passed by the Pennsylvania General Assembly on July 3, 2010, which established a new

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

inpatient hospital fee for service payment system, new supplemental payments and the waiver to establish the statewide Quality Care Assessment. DHS also received approval on final language for the DHS contracts with managed care organizations. The estimated net impact on the System for the years ended June 30, 2025, and 2024, was \$23,116 and \$18,983, respectively, (based on total payment increases of \$64,440 and \$52,496, respectively, offset by assessments of \$41,324 and \$33,513, respectively).

(c) *Nongovernmental Payors*

Inpatient services rendered by nongovernmental payors are reimbursed at negotiated rates. The System continues to be reimbursed for outpatient services at a negotiated percentage of covered charges.

(d) *Workers' Compensation*

The payment method by which all employers and/or insurers of workers' compensation policies will pay for the services provided by health care providers to employees covered by workers' compensation is a percentage of the Medicare payment for these services.

(e) *Other Contractual Arrangements*

The System has various payment agreements with preferred provider organizations and health maintenance organizations. The basis for payment under these agreements includes discounts from established charges.

The health care industry is subject to numerous laws and regulations of federal, state, and local governments. Compliance with these laws and regulations can be subject to future government review and interpretation as well as regulatory actions unknown or unasserted at the time. Recently, government activity has increased with respect to investigations and allegations concerning possible violations by health care providers of fraud and abuse statutes and regulations, which could result in the imposition of significant fines and penalties as well as significant repayments for patient services previously billed.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(4) Assets Whose Use is Limited and Investments

Assets whose use is limited and that are required for obligations classified as current liabilities are reported as current assets. The composition of assets whose use is limited at June 30, is set forth in the following tables.

	<u>2025</u>	<u>2024</u>
Self-insurance funding arrangements:		
Cash and cash equivalents	\$ 12,515	11,955
Certificates of deposit	24,472	30,110
U.S. government securities	2,857	5,495
Corporate and foreign bonds	2,244	2,443
Equity mutual funds	<u>1,307</u>	<u>1,135</u>
Total assets whose use is limited under self-insurance funding arrangements	\$ <u>43,395</u>	<u>51,138</u>
By board for capital improvements and under regulatory requirements:		
Cash and cash equivalents	\$ 12,502	12,843
Hedge, private equity, common collective trust funds	<u>51,845</u>	<u>60,679</u>
Total assets whose use is limited by the board for capital improvements and under regulatory requirements	\$ <u>64,347</u>	<u>73,522</u>
Investments with donor restrictions:		
Cash and cash equivalents	\$ 11,744	9,453
Equity mutual funds	23,926	20,745
Fixed income mutual funds	7,500	4,841
Beneficial interest in trusts	<u>23,107</u>	<u>21,434</u>
Total investments with donor restrictions	\$ <u>66,277</u>	<u>56,473</u>

The System's investments include a variety of financial instruments; the related values as presented in the consolidated financial statements are subject to various market fluctuations, which include changes in the equity markets, interest rate environment and general economic conditions.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

The following table presents fair value of assets whose use is limited as of June 30, 2025 and 2024:

	2025	2024
Cash and cash equivalents	\$ 36,761	34,251
Certificates of deposit	24,472	30,110
Corporate and foreign bonds	2,244	2,443
U.S. government securities	2,857	5,495
Equity mutual funds	25,233	21,880
Fixed income mutual funds	7,500	4,841
Hedge, private equity, common collective trust funds	51,845	60,679
Beneficial interests in trusts	23,107	21,434
Total	<u>\$ 174,019</u>	<u>181,133</u>

The following table represents the fair value measurement levels for all financial instruments, which the System has recorded at fair value on a recurring basis:

		2025		
	Fair value June 30, 2025	Quoted prices in active markets for identical assets (Level1)	Significant other observable inputs (Level2)	Significant other unobservable inputs (Level3)
Assets:				
Cash and cash equivalents	\$ 36,761	36,761	—	—
Certificates of deposit	24,472	24,472	—	—
Corporate and foreign bonds	2,244	—	2,244	—
U.S. government securities	2,857	—	2,857	—
Equity mutual funds	25,233	25,233	—	—
Fixed income mutual funds	7,500	7,500	—	—
Hedge, private equity, common collective trust funds (1)	51,845	—	—	—
Beneficial interest in trusts	23,107	1,086	—	22,021
Total assets whose use is limited	<u>\$ 174,019</u>	<u>95,052</u>	<u>5,101</u>	<u>22,021</u>

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

		2024			
		Fair value June 30, 2024	Quoted prices in active markets for identical assets (Level1)	Significant other observable inputs (Level2)	Significant other unobservable inputs (Level3)
Assets:					
Cash and cash equivalents	\$	34,251	34,251	—	—
Certificates of deposit		30,110	30,110	—	—
Corporate and foreign bonds		2,443	—	2,443	—
U.S. government securities		5,495	—	5,495	—
Equity mutual funds		21,880	21,880	—	—
Fixed income mutual funds		4,841	4,841	—	—
Hedge, private equity, common collective trust funds (1)		60,679	—	—	—
Beneficial interest in trusts		21,434	1,028	—	20,406
Total assets whose use is limited	\$	<u>181,133</u>	<u>92,110</u>	<u>7,938</u>	<u>20,406</u>

(1) Certain investments that are measured at net asset value per share (or its equivalent) as a practical expedient to fair value have not been categorized in the fair value hierarchy. The fair value amounts presented in this table are intended to permit reconciliation of the fair value hierarchy to the amounts presented in the consolidated balance sheets.

Following is the summary of the inputs and valuation techniques as of and for the years ended June 30, 2025 and 2024 for valuing Level 2 financial instruments:

Financial instrument	Input	Valuation technique
Corporate and foreign bonds	Broker/dealer	Market
U.S. government securities	Broker/dealer	Market

Transfers between levels occur when there is a change in the observability of significant inputs. A transfer between Level 1 and Level 2 generally occurs when the availability of quoted prices changes or when market activity of an investment significantly changes to active or inactive. A transfer between Level 2 and Level 3 generally occurs when the underlying inputs become, or can no longer be, corroborated with market observable data. Transfers between levels are recognized on the date they occur. For the years ended June 30, 2025 and 2024, no transfers were made between any Levels.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

The System holds instruments recorded at net asset value per share (or its equivalent) as a practical expedient to fair value and that do not have a readily determinable fair value as follows as of June 30:

<u>2025</u>	<u>Fair value</u>	<u>Unfunded commitments</u>	<u>Redemption frequency (if eligible)</u>	<u>Redemption notice period</u>
	(In millions)			
a – Multi-strategy hedge funds	\$ 2.4	—	Quarterly, annually	60 days
b – Real assets	0.1	0.1	N/A	N/A
c – Real estate funds	23.7	2.8	Annually	90 days
d – Private equity funds	25.7	7.6	N/A	N/A
Total	\$ 51.9	10.5		

<u>2024</u>	<u>Fair value</u>	<u>Unfunded commitments</u>	<u>Redemption frequency (if eligible)</u>	<u>Redemption notice period</u>
	(In millions)			
a – Multi-strategy hedge funds	\$ 2.9	—	Quarterly, annually	60 days
b – Real assets	0.1	0.3	N/A	N/A
c – Real estate funds	26.3	2.6	Annually	90 days
d – Private equity funds	31.3	8.5	N/A	N/A
Total	\$ 60.6	11.4		

- a. **Multi-Strategy Hedge Funds:** This class invests in hedge funds that pursue multiple strategies to diversify risks and reduce volatility. The fair values of the investments in this class have been estimated using the net asset value per share of the investments as a practical expedient. The remaining restriction period for these investments ranges from quarterly to annually.
- b. **Real Assets:** This class includes funds with direct investments in global and energy infrastructure. The fair values of the investments in this class have been estimated using the net asset value per share of the investments as a practical expedient.
- c. **Real Estate Funds:** This class includes real estate funds that invest in U.S. and non-U.S. residential and commercial properties as well as distressed real estate. The fair values of the investments in this class have been estimated using the net asset value of the System's ownership interest in partners' capital as a practical expedient. It is estimated that the underlying assets of these funds will be liquidated over the next 7 to 10 years, although these funds may liquidate early in the event of purchase by a third party or initial public offering. The fair values of the investments in this class have been estimated using the net asset value of the System's ownership interest in partners' capital as a practical expedient.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

- d. Private Equity Funds: This class includes private equity funds. These investments cannot be redeemed with the funds. Instead, the nature of the investments in this class is that distributions are received through the liquidation of the underlying assets of the fund. These funds are managed by one of the System's advisors with particular private equity experience in secondary market dealing. These funds could be subject to redemption to a third-party – buyer, but at June 30, 2025 and 2024, no funds were currently being evaluated this way. The fair values of the investments in this class have been estimated using the net asset value of the System's ownership interest in partners' capital as a practical expedient.

The System's use of Level 3 unobservable inputs at June 30, 2025 and 2024 accounted for 12.7% and 11.3%, respectively, of the total recurring fair value measurements. The Level 3 balances increased \$1,615 and \$866 for the years ended June 30, 2025 and 2024, respectively, due to increases in beneficial interest in trusts.

(5) Liquidity and Availability of Resources

Financial assets available within one year of the balance sheet date for general expenditures such as operating expenses and construction costs not financed with debt at June 30 are as follows:

	2025	2024
Cash and cash equivalents	\$ 135,437	91,056
Patient accounts receivable	236,560	220,299
Other receivables	38,500	40,529
Estimated third-party payor receivables	27,039	24,339
	<u>\$ 437,536</u>	<u>376,223</u>

Current financial assets not available for general use because of contractual or donor imposed restrictions totaled \$7,482 for the years ended June 30, 2025 and 2024. Amounts not available for general use include amounts set aside for self-insurance funds and perpetual, time and purpose donor restricted assets.

(6) Property, Plant and Equipment

Property, plant and equipment and related accumulated depreciation at June 30 consists of the following:

	2025	2024
Land and land improvements	\$ 44,458	48,283
Buildings and improvements	871,761	839,463
Fixed equipment	434,838	430,763

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

	<u>2025</u>	<u>2024</u>
Movable equipment (includes software and hardware)	\$ 717,236	672,548
Construction in progress	<u>38,300</u>	<u>91,195</u>
Property, plant and equipment before depreciation	2,106,593	2,082,252
Less accumulated depreciation	<u>(1,513,299)</u>	<u>(1,455,527)</u>
Property, plant and equipment, net	<u>\$ 593,294</u>	<u>626,725</u>

As of June 30, 2025, and 2024, assets under finance leases consist of medical buildings and equipment of \$6,264 and \$5,199 and accumulated amortization of \$5,930 and \$5,073, respectively. Assets under finance leases are included in property, plant and equipment on the accompanying consolidated balance sheets.

Depreciation expense relating to property, plant and equipment was \$65,066 and \$59,415 for the years ended June 30, 2025, and 2024, respectively.

At June 30, 2025 and 2024, the remaining commitments on construction in progress contracts are approximately \$2,632 and \$8,014, respectively.

(7) Long-term Debt

Long-term debt at June 30, 2025 and 2024 consists of the following:

	<u>2025</u>	<u>2024</u>
Berks County Municipal Authority Hospital Revenue Bond:		
Series of 2024, net of unamortized discount and premium	\$ 1,241,343	—
Berks County Municipal Authority Hospital Revenue Bond:		
Series of 2020, net of unamortized discount and premium	4,437	476,080
Berks County Industrial Development Authority Hospital Revenue Bond:		
Series of 2017, net of unamortized discount and premium	49,742	621,286
Berks County Municipal Authority Hospital Revenue Bond:		
Series of 2012, net of unamortized discount and premium	<u>33,313</u>	<u>164,400</u>
Total long-term debt	1,328,835	1,261,766
Less amounts due within one year	(50)	(9,355)
Less deferred financing costs, net	<u>(22,049)</u>	<u>(10,883)</u>
Long-term debt, net of current portion	<u>\$ 1,306,736</u>	<u>1,241,528</u>

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Scheduled principal repayments on long-term – debt are as follows for the years ending June 30:

2026	\$	50
2027		1,255
2028		55
2029		285
2030		80
Thereafter		<u>1,329,614</u>
Total long-term debt – par		1,331,339
Less unamortized net premiums/discounts		(2,504)
Less deferred financing costs, net		<u>(22,049)</u>
Long-term debt, net of unamortized premiums/discounts and deferred financing costs	\$	<u>1,306,786</u>

Tower Health System has borrowed funds through revenue bonds issued by the Berks County Municipal Authority (Municipal Authority) and the Berks County Industrial Development Authority (Development Authority). The proceeds originally were used in part to finance certain facilities of the System. The revenue bonds are secured by a pledge of revenue of the System. For accounting purposes, the revenue bonds are treated as though they are the debt of the entity which received the proceeds.

(a) Berks County Municipal Authority Hospital Revenue Bond Series of 2024

On September 19, 2024, the System completed an exchange offer and all outstanding bonds tendered were exchanged for new bonds. Total long-term debt as of September 30, 2024 was \$1,177,875, of which \$1,083,282 was exchanged.

For the new bonds, the System borrowed funds through new revenue bonds issued by the Municipal Authority. The revenue bonds are secured by a pledge of real assets and accounts receivable. For accounting purposes, the revenue bonds are treated as though they are the debt of the entity which received the proceeds. The new bonds consist of Series 2024A-1, 2024A-2, 2024A-3, 2024A-4, 2024B-1, and 2024B-2 Bonds, collectively Series 2024 Bonds.

The Series 2024 Bonds' interest payments are due in semi-annual installments beginning 2025 through 2044, with yearly aggregate payments ranging from \$34 to \$76. The 2024A Bonds are due in 2034 and 2039 with payments of \$157,500 and \$761,000, respectively. The 2024B Bonds are due in 2044 with a one-time payment of \$378,410. All Series 2024 are paid on fixed coupon rates between 5% and 8%

(b) Berks County Municipal Authority Hospital Revenue Bond Series of 2020

On February 11, 2020 the Municipal Authority issued \$264,595 of Revenue Bonds (2020) and \$190,720 Taxable Term Bonds for a total of \$455,315. Proceeds were used in the refunding the 2009 A-3 Bonds, Series 2012 B and C, Series 2016 A, C and D, Series 2017 A, funding the Interest

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Rate Swaps termination with the swap counterparty, the cost of issuance, and \$150,000 for general corporate purposes. The original issuance premium associated with the issuance was \$56,596.

At the time of issuance, the Series 2020 Bonds included \$40,910 of Serial Bonds, \$219,935 of Put Bonds, and \$190,720 of Taxable Bonds. The Serial and Put Bonds are due in annual installments beginning February 1, 2021 through February 1, 2040 with payments ranging from \$1,330 to \$20,910. The 2020 B-1 Series, which comprise \$64,565 of the Put Bonds, may be prepaid at any time after August 1, 2024 for a premium to par and may be tendered at par value beginning on February 1, 2025. The Taxable Term Bonds are due on February 1, 2050 with a one-time payment of \$190,720.

The effective interest rate at issuance ranges from 3.95% to 4.31% on the Serial Bonds and 4.45% on the Term Bonds.

All Series 2020 Series are paid at a fixed coupon rate between 4.45% and 5.00%. After the issuance of the Series 2020 Bonds and refunding of the Series mentioned, the System's long-term – debt portfolio is comprised of 100% fixed rate debt.

The Series 2020 Bonds were exchanged for a total \$427,752 of the Series 2024 Bonds leaving an outstanding debt balance of \$4,288. The exchange resulted in a gain of \$33,110, which is included within Gain on early extinguishment of debt in the accompanying statements of operations and changes in net (deficit) assets.

(c) Berks County Industrial Development Authority Hospital Revenue Bond Series of 2017

On October 31, 2017, the Development Authority issued \$590,500 of Revenue Bonds (2017) for the purpose of repaying a bridge loan and refunding the 2009 A-3 Bonds. The net original issuance premium associated with the issuance was \$56,151.

The Series 2017 Bonds are comprised of \$161,400 of Serial Bonds and \$429,100 of Term Bonds. The Serial Bonds are due in annual installments payable on November 1, 2021 through November 1, 2039 with payments ranging from \$1,770 to \$13,715. The Term Bonds are due on November 1, 2042, 2047 and 2050 with total payments of \$43,185, \$196,320, and \$189,595, respectively. The effective interest rate ranges from 4% to 5% on the Serial Bonds and 3.75% to 5% on the Term Bonds.

The Series 2017 Bonds were exchanged for a total of \$527,555 of the Series 2024 Bonds leaving an outstanding debt balance of \$52,190. The exchange resulted in a gain of \$34,404, which is included within Gain on early extinguishment of debt in the accompanying statements of operations and changes in net (deficit) assets.

(d) Berks County Municipal Authority Hospital Revenue Bond Series of 2012

On June 28, 2012, the Municipal Authority issued \$473,275 of Revenue Bonds in four Series, 2012 A, B, C, and D. At the end of June 30, 2025 and 2024, only the 2012 A remains outstanding.

The Municipal Authority issued \$160,065 of Fixed Rate Serial Revenue Bonds (2012 A) for the purpose of refunding the then existing Dauphin County General Authority Hospital Revenue Bond Series 1994A, and Berks County Bond Series 1998 and 2008. Mandatory annual principal redemptions by the System

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

for the 2012 A Bonds due November 1, 2039 through November 1, 2044, range from \$7,590 to \$33,555 with final maturity on November 1, 2044. Effective interest rate of the Bonds ranges from 4.23% to 4.50%.

The Series 2012 Bonds were exchanged for a total \$127,555 of the Series 2024 Bonds leaving an outstanding debt balance of \$32,510. The exchange resulted in a gain of \$2,565, which is included within Gain on early extinguishment of debt in the accompanying statements of operations and changes in net (deficit) assets.

(e) Line of Credit

The System secured a line of credit in May 2025 with Customers Bank providing available credit of \$35,000. Interest on any borrowings accrues at the Secured Overnight Financing Rate (SOFR) plus 2.75%. At June 30, 2025, the System balance outstanding on the line of credit was \$0 and the available borrowing capacity is \$35,000.

(f) Covenants

The various agreements place limits on the incurrence of additional borrowings and require that the System satisfy certain measures of financial performance as long as the debt is outstanding. These covenants apply to the System and include but are not limited to a long-term debt service coverage ratio of 1.0x, commencing September 30, 2025, and 30 days cash on hand, commencing June 30, 2025, measured at the end of each quarter. The agreements contain an excess cash flow provision that requires additional principal payments when days cash on hand is greater than 60 days. No excess cash payments are due as of June 30, 2025.

(8) Retirement Plans

During the year ended June 30, 2024, the System completed a retiree annuity purchase derisking strategy. The strategy settled future benefit commitments for 332 retirees with benefits below \$800 per month with Securian Financial for the year ended June 30, 2024. Securian agreed to administer payments to these former participants for their lifetimes, and their participation in the pension plan ceased effective July 1, 2024. The premium that the Reading Health System Pension Plan (the Plan) paid to Securian totaled \$24,470 reducing the pension liability by \$24,600. This amount was below the settlement threshold under ASC 715-30, so no one-time accounting was required for the transaction. There was no retiree annuity purchase during the year ended June 30, 2025. Obligations and plan assets at June 30 for the Plan:

	<u>2025</u>	<u>2024</u>
Change in projected benefit obligation:		
Benefit obligation at beginning of year	\$ 454,681	487,961
Interest cost	25,186	25,946
Actuarial gain	759	(8,409)
Benefits paid	<u>(25,054)</u>	<u>(50,817)</u>
Benefit obligation at end of year	\$ <u>455,572</u>	<u>454,681</u>

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

	<u>2025</u>	<u>2024</u>
Change in plan assets:		
Fair value of plan assets at beginning of year	\$ 404,555	402,432
Actual return on assets	39,016	38,910
Employer contributions	10,664	14,030
Benefits paid	<u>(25,054)</u>	<u>(50,817)</u>
Fair value of plan assets at end of year	\$ <u>429,181</u>	<u>404,555</u>

Benefits paid include \$24,470 for the retiree annuity purchase for the year ended June 30, 2024. No benefits were paid for the retiree annuity purchase for the year ended June 30, 2025.

Amounts recognized in the consolidated balance sheets at June 30 consist of:

	<u>2025</u>	<u>2024</u>
Accrued pension	\$ <u>26,391</u>	<u>50,126</u>
Total accrued liability	\$ <u><u>26,391</u></u>	<u><u>50,126</u></u>
Amounts recognized in net assets consist of:		
Net actuarial loss	\$ <u>142,344</u>	<u>155,604</u>
Pension cost charged to net assets without donor restrictions	\$ <u><u>142,344</u></u>	<u><u>155,604</u></u>

Net periodic pension cost (income) components at June 30 include the following:

	<u>2025</u>	<u>2024</u>
Interest cost on projected benefit obligation	\$ 25,187	25,946
Expected return on plan assets	(28,825)	(26,644)
Amortization of net loss	<u>3,829</u>	<u>4,530</u>
Net periodic pension cost	\$ <u><u>191</u></u>	<u><u>3,832</u></u>

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Other changes in plan assets and benefit obligations recognized in net assets without donor restrictions as of June 30:

	<u>2025</u>	<u>2024</u>
Net gain	\$ (9,432)	(20,675)
Amortization of net loss	<u>(3,829)</u>	<u>(4,530)</u>
Total recognized in net assets without donor restrictions	\$ <u>(13,261)</u>	<u>(25,205)</u>

Weighted average assumptions used to determine benefit obligations at June 30:

	<u>2025</u>	<u>2024</u>
Discount rate	5.70 %	5.67 %
Rate of compensation increase	N/A	N/A
Measurement date	6/30/2025	6/30/2024

Weighted average assumptions used to determine net periodic benefit cost for years ended June 30:

	<u>2025</u>	<u>2024</u>
Discount rate	5.67 %	5.47 %
Rate of compensation increase	7.25	6.75

To develop the expected long-term rate of return on assets assumption, the System considered the historical returns and the future expectations for returns for each asset class, as well as the target asset allocation of the pension portfolio.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(a) *Plan Assets*

The Plan's weighted average actual asset allocations and target allocations as of June 30 by asset category are as follows:

		2025	
		Target	Actual
Asset category:			
Cash and cash equivalents	— %		6.5 %
Equities, including mutual funds	56.0		38.0
Fixed income, including mutual funds, state, municipal government, and auction rate securities	30.0		31.5
Alternative investments (1)	14.0		24.0
	100.0 %		100.0 %
		2024	
		Target	Actual
Asset category:			
Cash and cash equivalents	— %		1.8 %
Equities, including mutual funds	56.0		46.1
Fixed income, including mutual funds, state, municipal government, and auction rate securities	30.0		26.2
Alternative investments (1)	14.0		25.9
	100.0 %		100.0 %

- (1) Long/Short Equity and Private Equity are classified as Alternative Investments. The overall investment objective of the Plan is to provide a return on investment consistent with the Plan's spending needs and to prevent erosion of purchasing power by inflation. Achievement of the return will be sought from an investment strategy that provides an opportunity for superior returns within acceptable levels of risk and volatility of returns. The following tables represent the fair value measurement levels for the Plan's investments.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

		2025		
		Quoted prices in active markets for identical assets (Level 1)	Significant other observable inputs (Level 2)	Significant other unobservable inputs (Level 3)
	Fair value June 30, 2025			
Cash and cash equivalents	\$ 27,907	27,907	—	—
Equity mutual funds	162,991	162,991	—	—
Equities	—	—	—	—
U.S. government securities	36,772	—	36,772	—
Corporate and foreign bonds	98,444	—	98,444	—
Hedge funds and private equity (1)	103,067	—	—	—
Total investments	\$ 429,181	190,898	135,216	—

		2024		
		Quoted prices in active markets for identical assets (Level 1)	Significant other observable inputs (Level 2)	Significant other unobservable inputs (Level 3)
	Fair value June 30, 2024			
Cash and cash equivalents	\$ 8,824	8,824	—	—
Equity mutual funds	173,776	173,776	—	—
Equities	12,031	12,031	—	—
U.S. government securities	36,577	—	36,577	—
Corporate and foreign bonds	68,895	—	68,895	—
Hedge funds and private equity (1)	104,452	—	—	—
Total investments	\$ 404,555	194,631	105,472	—

(1) Certain investments that are measured at net asset value per share (or its equivalent) as a practical expedient to fair value have not been categorized in the fair value hierarchy. The fair value amounts presented in this table are intended to permit reconciliation of the fair value hierarchy to the amounts presented in the consolidated balance sheets.

Transfers between levels occur when there is a change in the observability of significant inputs. A transfer between Level 1 and Level 2 generally occurs when the availability of quoted prices changes or when market activity of an investment significantly changes to active or inactive. A transfer between Level 2 and Level 3 generally occurs when the underlying inputs become, or can no longer be, corroborated with market observable data. Transfers between levels are recognized on the date they occur. For the years ended June 30, 2025 and 2024, no transfers were made between Levels.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

The Plan holds investments that calculate net asset value per share (or its equivalent) and do not have a readily determinable value are as follows as of June 30:

2025	Fair value	Unfunded commitments	Redemption frequency (if eligible)	Redemption notice period
	(In millions)			
a – Multi-strategy hedge funds	\$ 67.3	5.0	Quarterly and Annually	30-120 days
b – Real assets	3.3	1.7	N/A	N/A
c – Real estate funds	7.5	0.6	Annually	90 days
d – Private equity funds	25.0	2.5	N/A	N/A
Total	\$ 103.1	9.8		

2024	Fair value	Unfunded commitments	Redemption frequency (if eligible)	Redemption notice period
	(In millions)			
a – Multi-strategy hedge funds	\$ 61.0	3.2	Quarterly and Annually	30-120 days
b – Real assets	5.2	1.2	N/A	N/A
c – Real estate funds	8.2	1.3	Annually	90 days
d – Private equity funds	30.1	3.4	N/A	N/A
Total	\$ 104.5	9.1		

- a. Multi-Strategy: This class invests in hedge funds that pursue multiple strategies to diversify risks and reduce volatility. The fair values of the investments in this class have been estimated using the net asset value per share of the investments as a practical expedient. The remaining restriction period for these investments ranges from monthly to biennially.
- b. Real Assets: This class includes funds with direct investments in commodities and energy master limited partnerships. The fair values of the investments in this class have been estimated using the net asset value per share of the investments as a practical expedient.
- c. Real Estate Funds: This class includes real estate funds that invest in U.S. and non-U.S. residential and commercial properties as well as distressed real estate. The fair values of the investments in this class have been estimated using the net asset value of the System's ownership interest in partners' capital as a practical expedient. It is estimated that the underlying assets of these funds will be liquidated over the next 7 to 10 years, although these funds may liquidate early in the event of purchase by a third party or initial public offering.
- d. Private Equity Funds: This class includes private equity funds. These investments cannot be redeemed with the funds. Instead, the nature of the investments in this class is that distributions are received through the liquidation of the underlying assets of the fund. These funds could be subject to redemption to a third-party buyer, but at June 30, 2025 and 2024, no funds were currently being evaluated this way. The fair values of the investments in this class have been

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

estimated using the net asset value of the System's ownership interest in partners' capital as a practical expedient.

(b) Contributions

The System expects to contribute the minimum required contribution during the fiscal year 2025 to the Plan, which is estimated to be \$13,630. For the years ended June 30, 2025 and 2024, the System contributed \$10,664 and \$14,030, respectively to the Plan. For the years ended June 30, 2025 and 2024, the System contributed \$26,506 and \$25,981, respectively to the defined contribution plan (403b) and \$1,779 and \$1,736, respectively to the nonqualified deferred compensation plan (457f).

(c) Estimated Future Benefit Payments

The following Plan benefit payments are expected to be paid for the fiscal years ending June 30:

2026	\$	26,876
2027		28,132
2028		29,287
2029		30,281
2030		31,118
Thereafter		162,536
Total	\$	<u>308,230</u>

(9) Net Assets with Donor Restrictions

Net assets with donor restrictions are available for the following purposes at June 30:

	<u>2025</u>	<u>2024</u>
Various health care services	\$ 29,907	27,775
Permanent endowment funds, the interest and dividend income from which is expendable to support health care services	<u>23,110</u>	<u>21,433</u>
Total donor restricted net assets	<u>\$ 53,017</u>	<u>49,208</u>

(10) Insurance Arrangements

The System participates in the Pennsylvania Medical Care Availability and Reduction of Error Fund or Mcare Fund established under the Commonwealth of Pennsylvania. For each hospital, the Mcare Fund provides coverage of up to \$500 per occurrence and annual aggregate coverage of \$1,500 in excess of each hospital's primary limit of \$500 per occurrence and annual aggregate coverage of \$2,500. Mcare Fund eligible providers and the Reading Hospital Transitional Subacute Unit (TSU) are also afforded Mcare Fund coverage of up to \$500 and annual aggregate coverage of \$1,500 in excess of each provider's and the TSU's primary limits of \$500 per occurrence and \$1,500 annual aggregate.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

The System established a self-insurance trust fund to provide protection against professional liability claims for the legacy Reading hospital and its affiliates for occurrence prior to October 1, 2018. The trust is actuarially funded on an annual basis to provide single limit professional liability coverage of \$500 per occurrence and \$2,500 in the annual aggregate for the Hospital and certain employees. For incidents occurring since April 30, 2009, the System purchased commercial insurance to provide coverage on a claims made basis. The amount up in of excess available above the total self-retention and Mcare excess changed over the course of years based on the markets and the System's liabilities. Claim liabilities are presented gross of any insurance recoveries. The system was self-insured for general liability with limits of \$1,000 per occurrence and \$3,000 annual aggregate. Certain claim liabilities are discounted at an interest rate of 3% for the years ended June 30, 2025, and 2024 and decreased the undiscounted liability as of June 30, 2025, and 2024 by \$4,069 and \$955, respectively. For the years ended June 30, 2025, and 2024, the insurance recoverable amount was \$30,459 and \$41,660, respectively, which is included in other receivables and other assets on the consolidated balance sheets. Funding requirements of the plan are subject to increase depending on the plan's claim experience. Premium payments for the Mcare Fund are based upon each individually licensed healthcare provider's rating with the Joint Underwriters Association and the amount of the surcharge to be assessed is determined by the Mcare Fund on an annual basis. The System's annual surcharge premium for participation in the Mcare Fund was \$4,106 and \$2,812 for the years ended June 30, 2025, and 2024, respectively.

During the year ended June 30, 2018, in conjunction with the acquisition of CMP, Tower Health formed a captive Reciprocal Risk Retention Group (RRG) subsidiary for the purpose of self-insuring – malpractice at CMP. During the year ended June 30, 2019, a decision was made to extend the RRG coverage not only to the CMP professional liabilities, but to cover Reading Hospital and all affiliates as well. All reserves and liabilities accrued to Reading Hospital will be borne by the existing self-insurance – coverages and, beginning in the second quarter of fiscal 2019, all new reserves incurred, and liabilities will accrue to the RRG. Premiums in the amount of \$11,680 and \$11,179 were paid for the years June 30, 2025 and 2024. Statutory funding of \$4,500 and \$2,000 is included in assets under regulatory requirements on the consolidated balance sheets for the years ended June 30, 2025 and 2024.

Additionally, the System self-insures its workers' compensation and minor general liability risks. The System's self-insurance plan has been reviewed and approved by the Commissioner of Insurance of Pennsylvania. The System purchases excess workers' compensation insurance for all controlled entities of the System with statutory limits over a self-retention of \$1,000 per occurrence subject to a policy maximum of \$1,000 for the policy period. Workers' compensation liabilities are discounted at an interest rate of 3% for the years ended June 30, 2025 and 2024 and decreased the undiscounted liability as of June 30, 2025 and 2024 by \$1,285 and \$1,339, respectively.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Reserve for self-insurance claims at June 30 are summarized as follows:

	<u>2025</u>	<u>2024</u>
Professional liability claims payable	\$ 109,279	110,260
Workers' compensation	<u>7,532</u>	<u>8,487</u>
Total self-insurance claims reserve	116,811	118,747
Less current portion	<u>(28,563)</u>	<u>(49,751)</u>
Self-insurance claims reserve, net of current portion	\$ <u>88,248</u>	<u>68,996</u>

(11) Commitments and Contingencies

(a) Leases

Total operating lease cost was \$27,405 and \$34,834 for the years ended June 30, 2025 and 2024, respectively. Commitments relating to noncancellable operating, and finance leases obligations for each of the next five years and thereafter are as follows:

	<u>Finance leases</u>	<u>Operating leases</u>
2026	\$ 4,765	24,458
2027	4,620	22,743
2028	4,183	21,719
2029	3,121	19,931
2030	4,394	18,956
Thereafter	<u>2,848</u>	<u>237,704</u>
Total future minimum lease payments	23,931	345,511
Less amount representing interest	<u>(2,975)</u>	<u>(104,412)</u>
Total finance and operating leases	20,956	241,099
Less current position	<u>(4,765)</u>	<u>(24,458)</u>
Total finance and operating leases, net of current portion	\$ <u>16,191</u>	<u>216,641</u>

The finance leases represent the present value of future minimum lease payments, bear imputed interest at 4.30% and mature at dates ranging from 2028 to 2031.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Supplemental cash flow and other information related to operating leases as of and for the years ended June 30 are as follows:

	<u>2025</u>	<u>2024</u>
Cash paid for amounts included in the measurement of lease liabilities:		
Operating cash flows from operating leases	\$ 23,442	26,971
Right-of-use assets obtained in exchange for new operating lease liabilities	1,065	10,088
Weighted average remaining lease term (in years):		
Operating leases	21.9	22.4
Weighted average, discount rate:		
Operating leases	4.3 %	4.3 %

(b) Financing Obligations

In June 2020, the System entered into sale-leaseback – transactions with a third party which did not qualify as a sale. As a result of the transactions, (1) the assets and accumulated depreciation remain on the consolidated balance sheet and continue to be depreciated over the remaining useful lives; (2) no gain is recognized; and (3) proceeds received from these transactions are recorded as financing obligations.

Financing obligations represent the present value of minimum lease payments under such lease arrangements and bear interest of approximately 4.3% at both June 30, 2025 and 2024, and the initial term maturity dates range from 2035 to 2040.

The following table summarizes the future minimum payments for the next five years and thereafter under leases classified as financing obligations for the fiscal years ending June 30:

2026	\$ 12,721
2027	12,975
2028	13,235
2029	13,500
2030	13,770
Thereafter	<u>134,857</u>
Total future minimum lease payments	\$ 201,058

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

Less amount representing interest	\$ <u>(55,523)</u>
Total financing obligations	145,535
Less current portion	(12,721)
Less deferred financing costs	<u>(4,859)</u>
Total financing obligations, net of current	\$ <u>127,955</u>

(c) Drexel University

In February 2019, the System and Drexel University signed a 20-year academic agreement. In connection with this affiliation, a new four-year regional campus of Drexel University College of Medicine was constructed in West Reading, PA.

(d) Litigation

The System and its controlled entities are involved in certain litigation, which involves professional and general liability. In the opinion of management and legal counsel, the ultimate liability, if any, will not have a material effect on the consolidated financial condition of the Parent and its subsidiaries.

(e) Regulatory Compliance

Laws and regulations governing the Medicare and Medicaid programs are complex and subject to interpretation. Compliance with such laws and regulations can be subject to government review and interpretation as well as significant regulatory action, including fines, penalties, and exclusion from the Medicare and Medicaid programs.

(13) Concentrations

(a) Concentrations of Credit Risk

Financial instruments, which potentially subject the System to concentrations of credit risk, consist primarily of cash, cash equivalents, investments, and accounts receivable.

Management periodically evaluates the credit standing of the financial institutions with which the System maintains its cash, cash equivalents, and investments. Amounts held in its accounts often exceed the federally insured levels.

The fair value of the System's investments is subject to various market fluctuations, which include changes in the interest rate environment and general economic conditions.

(b) Unions and Collective Bargaining

As of June 30, 2025 and 2024, approximately 5.3% and 5.2% of the System's employees are subject to collective bargaining agreements with various unions, respectively. The bargaining agreements have various expiration dates, with the next expiration in 2027.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

(14) Functional Expenses

Expenses attributed to each program or supporting function of the System are reported in the following tables. Expenses attributable to more than one program require allocation, which is consistently applied and based upon reasonable statistics such as revenue, expenses, or full-time equivalents. The System considers health program services and general/administrative to be its primary functional categories for purposes of expense classification. General/administrative includes information systems, general corporate management, advertising, and marketing. Functional categories of expenses, excluding impairment, for the years ended June 30 are as follows:

2025			
	Healthcare services	General and administrative	Total
Salaries and benefits	\$ 924,236	92,568	1,016,804
Supplies	412,509	513	413,022
Interest	66,113	—	66,113
Depreciation	57,668	7,398	65,066
Purchased services	242,757	28,983	271,740
Repairs and maintenance	20,963	35,181	56,144
Other	112,874	48,078	160,952
Total	<u>\$ 1,837,120</u>	<u>212,721</u>	<u>2,049,841</u>

2024			
	Healthcare services	General and administrative	Total
Salaries and benefits	\$ 947,346	117,546	1,064,892
Supplies	349,958	645	350,603
Interest	55,475	—	55,475
Depreciation	51,062	8,353	59,415
Purchased services	147,558	55,138	202,696
Repairs and maintenance	25,943	39,226	65,169
Other	130,135	46,288	176,423
Total	<u>\$ 1,707,477</u>	<u>267,196</u>	<u>1,974,673</u>

(15) Significant Risks and Uncertainties

The Patient Protection and Affordable Care Act and the Health Care and Education Reconciliation Act of 2010 (collectively, Health Care Reform), enacted in March 2010, have changed and will continue to make broad-based changes to the U.S. health care system which could significantly affect the U.S. economy and which the System expects will continue to impact the System's business operations and financial results. Since its enactment in 2010, key components of Health Care Reform have been phased in, including health insurance exchanges (Public Exchanges), new Medicare products, and the individual coverage mandate. Although Health Care Reform was phased in through 2018, many significant changes occurred in 2014.

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

The System is dedicating material resources to monitor the potential impacts of Health Care Reform as well as state level health care reform. While the federal government has issued a number of regulations implementing Health Care Reform, certain significant parts of Health Care Reform, including aspects of Public Exchanges, Medicaid expansion, enforcement related reporting for the individual and employer mandates, and the implementation of Medicare Advantage, require further guidance and clarification at the federal level and/or in the form of regulations and actions by state legislatures to implement the law. The federal government also has announced significant changes to and/or delays in effective dates of various aspects of Health Care Reform, and it is likely that further changes will be made at the federal and/or state level based on implementation experience. As a result, key aspects and impacts of Health Care Reform will not be known for several years and given the inherent difficulty of foreseeing how individuals and businesses will respond to the choices afforded them by Health Care Reform, the System cannot predict the full effect Health Care Reform will have on the System. It is reasonably possible that Health Care Reform, in the aggregate, could have an adverse effect on the System's business operations and financial results.

Federal budget negotiations, ongoing regulatory changes to Health Care Reform, pending efforts in the U.S. Congress to amend or restrict funding for various aspects of Health Care Reform and litigation challenging aspects of the law continue to create uncertainty about the ultimate impact of Health Care Reform.

In addition, the federal and state governments continue to enact or seriously consider many other broad-based legislative and regulatory proposals that have impacted or could materially impact various aspects of the health care system. The System cannot predict whether pending or future federal or state legislation will change various aspects of the health care system or Health Care Reform or the impact those changes will have on the System's business operations or financials results, but the effects could be adverse.

The System recognizes the increasing importance of cybersecurity in today's digital landscape. As a result, the System has implemented various measures to mitigate the risk of cyber threats and protect our systems and data as well as monitor the risks that our vendors have. However, we understand that no system is completely immune to cyberattacks, and there is a possibility that an unauthorized access, data breach, or other cybersecurity incident may occur at either one of our systems or at one of vendors' systems. In the event of a significant cyber incident, there could be a significant impact to the System's future operating results, financial condition, or liquidity. However, to mitigate the potential impact to the System if such an event were to occur, the System maintains cyber insurance coverage. While we believe our cybersecurity measures and our vendors' measures are robust, there can be no assurance that they will prevent all cyber threats or that there will not be a cyber incident in the future that may not have a significant adverse effect on our financial condition, liquidity, or results of operations.

(16) Related Party Transactions

Members of the board, officers, and employees are subject to the System's conflict of interest policies, under which business and financial relationships must be disclosed and are subject to review and approval. One board member of Tower Health and one board member of Tower Health at Home are partners and/or shareholders of a legal firm that provides legal services as it relates to day-to-day operations of Tower Health. The respective board members of Tower Health do not perform services for the System. The legal

TOWER HEALTH AND SUBSIDIARIES

Notes to Consolidated Financial Statements

June 30, 2025 and 2024

(Dollars in thousands)

fees paid to the law firm were \$5,703 and \$6,309 for the years ended June 30, 2025 and 2024, respectively.

The Parent and Drexel University guarantee the STC real estate lease from a third-party landlord. The System has entered into a management agreement with STC to provide certain back-office, administrative, project management, clinical and information technology services. Management fee revenues from STC were \$12,137 and \$18,716 for the years ended June 30, 2025 and 2024, respectively, which are included in other operating revenue in the accompanying consolidated statement of operations and changes in net (deficit) assets. Interest income on the line of credit to STC for the years ended June 30, 2025, and 2024 was \$3,573 and \$3,584, respectively, which is included in other operating revenue in the accompanying consolidated statement of operations and changes in net (deficit) assets.

(17) Subsequent Events

The System has evaluated subsequent events from the consolidated balance sheet date through November 7, 2025.

On February 28, 2023, the System and CG Broadcasting, LLC entered into a purchase and sale agreement for a parcel of land totaling 103.77 acres located in Spring Township, PA. The sale proceeds received was \$23,500 and the transaction closed September 26, 2025.

TOWER HEALTH AND SUBSIDIARIES
Schedule of Expenditures of Federal Awards
Year ended June 30, 2025

Federal grantor/program or cluster title	Federal assistance listing number (ALN)	Pass-thru entity identification number	Federal expenditures
Student Financial Assistance Cluster:			
U.S. Department of Education:			
Federal Direct Student Loans	84.268		\$ 1,157,397
Federal Pell Grant Program	84.063		669,397
Total Student Financial Assistance Cluster			<u>1,826,794</u>
Research and Development Cluster:			
U.S. Department of Health and Human Services:			
Diabetes, Digestive, and Kidney Diseases Extramural Research			
Passed through Johns Hopkins University	93.847	2005279338	66,309
Lung Diseases Research			
Passed through Johns Hopkins University	93.838	2006629433	3,000
Cardiovascular Diseases Research			
Passed through University of Rochester	93.837	SUB00000244AM3 / UR FAO GR531832	2,222
Total Research and Development Cluster			<u>71,531</u>
Other programs:			
U.S. Department of Treasury:			
COVID-19: Coronavirus State and Local Fiscal Recovery Funds			
Passed through Montgomery County, Pennsylvania	21.027	SLFRP3241	424,870
U.S. Department of Health and Human Services:			
Grants to Provide Outpatient Early Intervention Services with Respect to HIV Disease (Ryan			
White HIV/AIDS Program Part C)	93.918		315,043
Research on Research Integrity	93.085		27,670
Block Grants for the Prevention and Treatment of Substance Abuse - Soft Landing			
Passed through the Pennsylvania Council on Chemical Abuse	93.959	067095	293,167
Total U.S. Department of Health and Human Services			635,880
U.S. Department of Homeland Security			
COVID-19: Disaster Grants - Public Assistance (Presidentially Declared Disasters)			
Passed through the Commonwealth of Pennsylvania Emergency Management Agency	97.036	011-URM0S-00	409,051
Total other programs			<u>1,469,801</u>
Total expenditures of federal awards			<u>\$ 3,368,126</u>

See accompanying notes to schedule of expenditures of Federal awards.

TOWER HEALTH AND SUBSIDIARIES

Notes to Schedule of Expenditures of Federal Awards

Year ended June 30, 2025

(1) Basis of Presentation

The accompanying Schedule of Expenditures of Federal Awards (the Schedule) includes the federal award activities of Tower Health and its subsidiaries (the System) under programs of the federal government for the year ended June 30, 2025. The School of Health Sciences (the School) provides a program of classroom and clinical preparation for the nursing profession and other allied health professionals. The information in this Schedule is presented in accordance with the requirements of Title 2 U.S. Code of Federal Regulations Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (Uniform Guidance). The Schedule presents only a selected portion of the operations of the System, and it is not intended to and does not present the consolidated financial position, changes in net assets, or cash flows of the System.

There were no federal funds passed through to subrecipients for the year ended June 30, 2025.

(2) Summary of Significant Accounting Policies

Expenditures reported on the Schedule, except for the program expenditures described in the following paragraph, are reported on the accrual basis of accounting. Such expenditures are recognized following the cost principles contained in the Uniform Guidance, or 45 CFR Part 75 Appendix IX, *Principles for Determining Costs Applicable to Research and Development Under Grants and Contracts with Hospitals*, wherein certain types of expenditures are not allowable or are limited as to reimbursement. Therefore, some amounts presented in the Schedule may differ from amounts presented in, or used in the preparation of, the consolidated financial statements.

Expenditures incurred in prior years reported on the Schedule for certain grants are recognized based on the following:

- Disaster Grants - Public Assistance (Presidentially Declared Disasters), federal assistance listing number 97.036, represents reportable expenditures when both of the following criteria are met (1) the Federal Emergency Management Agency has approved the System's project, and (2) the eligible expenditures have been incurred. During the year ended June 30, 2025, the Pennsylvania Emergency Management Agency sub-awarded to the System federal funding obligated in 2025 by the U.S. Department of Homeland Security through the Federal Emergency Management Agency totaling \$409,051 for various project expenditures that were incurred in prior years.

(3) Indirect Costs

The System has not elected to use the 10-percent de minimis indirect cost rate allowed under the Uniform Guidance.

(4) Federal Direct Student Loans Program

With respect to the Federal Direct Student Loans Program, the School is only responsible for the performance of certain administrative duties; therefore, the transactions and balances of loans outstanding related to this program are not included in the System's consolidated financial statements. The Schedule includes the amounts loaned to students during the year ended June 30, 2025. It is not practical to estimate the outstanding balance of loans under this program.

PART II
REPORTS ON INTERNAL CONTROL AND COMPLIANCE



KPMG LLP
Suite 4000
1735 Market Street
Philadelphia, PA 19103-7501

**Independent Auditors' Report on Internal Control Over Financial Reporting and on
Compliance and Other Matters Based on an Audit of Financial Statements Performed in
Accordance With Government Auditing Standards**

The Board of Directors
Tower Health:

We have audited, in accordance with the auditing standards generally accepted in the United States of America and the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States, the consolidated financial statements of Tower Health and its subsidiaries (the System), which comprise the System's consolidated balance sheets as of June 30, 2025, and the related consolidated statements of operations and changes in net (deficit) assets, and cash flows for the year then ended, and the related notes to the consolidated financial statements, and have issued our report thereon dated November 7, 2025.

Report on Internal Control Over Financial Reporting

In planning and performing our audit of the consolidated financial statements, we considered the System's internal control over financial reporting (internal control) as a basis for designing audit procedures that are appropriate in the circumstances for the purpose of expressing our opinion on the consolidated financial statements, but not for the purpose of expressing an opinion on the effectiveness of the System's internal control. Accordingly, we do not express an opinion on the effectiveness of the System's internal control.

A deficiency in internal control exists when the design or operation of a control does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, misstatements on a timely basis. A material weakness is a deficiency, or a combination of deficiencies, in internal control, such that there is a reasonable possibility that a material misstatement of the entity's financial statements will not be prevented, or detected and corrected, on a timely basis. A significant deficiency is a deficiency, or a combination of deficiencies, in internal control that is less severe than a material weakness, yet important enough to merit attention by those charged with governance.

Our consideration of internal control was for the limited purpose described in the first paragraph of this section and was not designed to identify all deficiencies in internal control that might be material weaknesses or significant deficiencies. Given these limitations, during our audit we did not identify any deficiencies in internal control that we consider to be material weaknesses. However, material weaknesses or significant deficiencies may exist that were not identified.

Report on Compliance and Other Matters

As part of obtaining reasonable assurance about whether the System's consolidated financial statements are free from material misstatement, we performed tests of its compliance with certain provisions of laws, regulations, contracts, and grant agreements, noncompliance with which could have a direct and material effect on the financial statements. However, providing an opinion on compliance with those provisions was not an objective of our audit, and accordingly, we do not express such an opinion. The results of our tests disclosed no instances of noncompliance or other matters that are required to be reported under *Government Auditing Standards*.



Purpose of This Report

The purpose of this report is solely to describe the scope of our testing of internal control and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the System's internal control or on compliance. This report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the entity's internal control and compliance. Accordingly, this communication is not suitable for any other purpose.

KPMG LLP

Philadelphia, Pennsylvania
November 7, 2025



KPMG LLP
Suite 4000
1735 Market Street
Philadelphia, PA 19103-7501

**Independent Auditors' Report on Compliance for Each Major Federal Program;
Report on Internal Control Over Compliance; and Report on Schedule of
Expenditures of Federal Awards Required by the Uniform Guidance**

The Board of Directors
Tower Health:

Report on Compliance for Each Major Federal Program

Opinion on Each Major Federal Program

We have audited Tower Health and its subsidiaries' (the System) compliance with the types of compliance requirements identified as subject to audit in the *OMB Compliance Supplement* that could have a direct and material effect on the System's major federal program for the year ended June 30, 2025. The System's major federal program is identified in the summary of auditors' results section of the accompanying schedule of findings and questioned costs.

In our opinion, Tower Health and its subsidiaries complied, in all material respects, with the compliance requirements referred to above that could have a direct and material effect on its major federal program for the year ended June 30, 2025.

Basis for Opinion on Each Major Federal Program

We conducted our audit of compliance in accordance with auditing standards generally accepted in the United States of America (GAAS); the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States; and the audit requirements of Title 2 U.S. Code of Federal Regulations Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (Uniform Guidance). Our responsibilities under those standards and the Uniform Guidance are further described in the Auditors' Responsibilities for the Audit of Compliance section of our report.

We are required to be independent of the System and to meet our other ethical responsibilities, in accordance with relevant ethical requirements relating to our audit. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion on compliance for each major federal program. Our audit does not provide a legal determination of the System's compliance with the compliance requirements referred to above.

Responsibilities of Management for Compliance

Management is responsible for compliance with the requirements referred to above and for the design, implementation, and maintenance of effective internal control over compliance with the requirements of laws, statutes, regulations, rules and provisions of contracts or grant agreements applicable to the System's federal programs.

Auditors' Responsibilities for the Audit of Compliance

Our objectives are to obtain reasonable assurance about whether material noncompliance with the compliance requirements referred to above occurred, whether due to fraud or error, and express an opinion on the System's compliance based on our audit. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS,



Government Auditing Standards, and the Uniform Guidance will always detect material noncompliance when it exists. The risk of not detecting material noncompliance resulting from fraud is higher than for that resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Noncompliance with the compliance requirements referred to above is considered material if there is a substantial likelihood that, individually or in the aggregate, it would influence the judgment made by a reasonable user of the report on compliance about the System's compliance with the requirements of each major federal program as a whole.

In performing an audit in accordance with GAAS, *Government Auditing Standards*, and the Uniform Guidance, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material noncompliance, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the System's compliance with the compliance requirements referred to above and performing such other procedures as we considered necessary in the circumstances.
- Obtain an understanding of the System's internal control over compliance relevant to the audit in order to design audit procedures that are appropriate in the circumstances and to test and report on internal control over compliance in accordance with the Uniform Guidance, but not for the purpose of expressing an opinion on the effectiveness of the System's internal control over compliance. Accordingly, no such opinion is expressed.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and any significant deficiencies and material weaknesses in internal control over compliance that we identified during the audit.

Report on Internal Control Over Compliance

A deficiency in internal control over compliance exists when the design or operation of a control over compliance does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, noncompliance with a type of compliance requirement of a federal program on a timely basis. A material weakness in internal control over compliance is a deficiency, or a combination of deficiencies, in internal control over compliance, such that there is a reasonable possibility that material noncompliance with a type of compliance requirement of a federal program will not be prevented, or detected and corrected, on a timely basis. A significant deficiency in internal control over compliance is a deficiency, or a combination of deficiencies, in internal control over compliance with a type of compliance requirement of a federal program that is less severe than a material weakness in internal control over compliance, yet important enough to merit attention by those charged with governance.

Our consideration of internal control over compliance was for the limited purpose described in the Auditors' Responsibilities for the Audit of Compliance section above and was not designed to identify all deficiencies in internal control over compliance that might be material weaknesses or significant deficiencies in internal control over compliance. Given these limitations, during our audit we did not identify any deficiencies in internal control over compliance that we consider to be material weaknesses, as defined above. However, material weaknesses or significant deficiencies in internal control over compliance may exist that were not identified.

Our audit was not designed for the purpose of expressing an opinion on the effectiveness of internal control over compliance. Accordingly, no such opinion is expressed.

The purpose of this report on internal control over compliance is solely to describe the scope of our testing of internal control over compliance and the results of that testing based on the requirements of the Uniform Guidance. Accordingly, this report is not suitable for any other purpose.



Report on Schedule of Expenditures of Federal Awards Required by the Uniform Guidance

We have audited the consolidated financial statements of Tower Health and its subsidiaries as of and for the year ended June 30, 2025, and have issued our report thereon dated November 7, 2025, which contained an unmodified opinion on those consolidated financial statements. Our audit was performed for the purpose of forming an opinion on the consolidated financial statements as a whole. The accompanying schedule of expenditures of federal awards is presented for purposes of additional analysis as required by the Uniform Guidance and is not a required part of the consolidated financial statements. Such information is the responsibility of management and was derived from and relates directly to the underlying accounting and other records used to prepare the consolidated financial statements. The information has been subjected to the auditing procedures applied in the audit of the consolidated financial statements and certain additional procedures, including comparing and reconciling such information directly to the underlying accounting and other records used to prepare the consolidated financial statements or to the consolidated financial statements themselves, and other additional procedures in accordance with GAAS. In our opinion, the schedule of expenditures of federal awards is fairly stated in all material respects in relation to the consolidated financial statements as a whole.

KPMG LLP

Philadelphia, Pennsylvania
February 19, 2026

PART III FINDINGS

TOWER HEALTH AND SUBSIDIARIES
Schedule of Findings and Questioned Costs
Year ended June 30, 2025

(1) Summary of Auditor's Results

- (a) Type of report issued on whether the consolidated financial statements were prepared in accordance with U.S. generally accepted accounting principles: **Unmodified**
- (b) Internal control deficiencies over financial reporting disclosed by the audit of the consolidated financial statements:
 - Material weaknesses: **No**
 - Significant deficiencies: **None reported**
- (c) Noncompliance material to the consolidated financial statements: **No**
- (d) Internal control deficiencies over the major program disclosed by the audit:
 - Material weaknesses: **No**
 - Significant deficiencies: **None reported**
- (e) Type of report issued on compliance for the major program: **Unmodified**
- (f) Audit findings that are required to be reported in accordance with 2 CFR 200.516(a): **No**
- (g) Major program:
 - Student Financial Assistance Cluster: ALNs 84.268 and 84.063
- (h) Dollar threshold used to distinguish between Type A and Type B programs: **\$750,000**
- (i) Auditee qualified as a low-risk auditee: **Yes**

(2) Findings Relating to the Consolidated Financial Statements Reported in Accordance with Government Auditing Standards

None

(3) Findings and Questioned Costs Relating to Federal Awards

None